June 16, 2025

Centers for Medicare & Medicaid Services
Department of Health and Human Services

RE: Response to CMS Request for Information: Health Technology Ecosystem (CMS–6082–NC)

As a developer of digital diagnostic solutions, BioBeat Technologies is pleased to submit this response focused on the intersection of reimbursement policy, digital health access, and clinical workflow modernization.

Hypertension is one of the most prevalent and costly medical conditions in the US, with nearly half of all U.S. adults diagnosed with hypertension and annual costs estimated around $220 billion annually in 2019, per the CDC. Three-quarters of people with hypertension do not have their blood pressure under control, leading to almost 700,000 deaths in 2022 in which hypertension was the primary or contributing cause of death. Effective and highly cost-efficient options to diagnose and manage BP are available, including ABPM, but adoption is very low. For instance, self-monitoring of blood pressure is estimated to save $7.50 to $20 for every $1 invested over 10 years, yet far fewer than 0.01% of people with hypertension use ABPM technology despite national coverage. Streamlining access to ABPM services – and ensuring coverage of more convenient and manageable ABPM technology – can dramatically improve long-term Medicare costs and health outcomes

About BioBeat and Our Technology

BioBeat USA Inc. offers a modern cuff-less, wearable FDA-cleared device that accurately provides ambulatory BP monitoring based on photoplethysmography (PPG) and other validated methodologies that can reliably monitor blood pressure over 24-hour cycles without the discomfort, complexity, and operational barriers posed by traditional cuff-based systems.

As we explained when we were honored to present at Mar-a-Lago during US-ISRAEL AI FORUM on May 7, 2025, BioBeat's wearable sensor is a chest-applied, fully cuffless device that captures blood pressure every 15 minutes over 24 hours. The device passively transmits the data via a mobile app to a secure cloud, where it is processed into a comprehensive clinical report viewable by the provider through our web portal. BioBeat's system requires no ongoing patient interaction, making it especially well-suited for older adults and patients with limited digital literacy.

Response to RFI Questions

PC-6: Features for Accessibility and Ease of Use

Key features that improve accessibility for Medicare beneficiaries and caregivers, especially those with limited digital experience, include:

- Passive data capture: Once placed, BioBeat's device collects blood pressure without any patient action.
- Minimal setup: The device uses a pre-configured mobile app, requiring no complex patient use
- Wearable comfort: Its cuffless form factor eliminates the discomfort and anxiety associated with traditional cuffs.

- Provider-mediated use: Data flows only to the physician, relieving patients of interpretation burdens.
- Simple visuals and language: Any user-facing interface uses large text, icons, and clear instructions.

These design choices reduce friction and ensure high adherence even among older or tech-averse populations.

C. Providers

1. Digital Health Apps

PR-1. What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients?

CMS and its partners can take the following actions in order to leverage approved digital health products for their patients:

Today, the Medicare NCD covering Ambulatory Blood Pressure Monitoring (ABPM) requires that ABPM devices be "provided to patients with oral and written instructions, and a test run in the physician's office must be performed." As CMS has allowed in the context of ambulatory heart rhythm monitoring, home sleep testing, and remote physiological monitoring, devices for self-monitoring can be provided to the patient in their home, with device initiation conducted by video, telemedicine, or over the phone under the general supervision of the ordering or performing physician. An in-person visit to the physician's office to apply a device and run a test is not only unnecessary, but it burdens physician clinics and their staff – along with patients – with an administrative visit without providing any benefit.

We recommend CMS clarify that "in the physician's office" should be interpreted to mean "under the general supervision of a physician," which can allow for remote delivery of ABPM devices and services, consistent with other types of ambulatory monitoring. Allowing remote patient self-setup with telemedicine support will:

- Align ABPM services with the broader expansion of remote care delivery CMS has embraced in other areas.
- Reduce patient travel and increase compliance among those with transportation or mobility limitations.
- Decrease operational burden and clinic overhead, freeing clinical resources for more acute patient needs.

In addition, to facilitate access in rural areas, CMS must provide adequate reimbursement for ABPM services in physician clinic settings (non-hospitals) that are more accessible. The relevant CPT codes 93784, 93786,93788 and 93790

As of today, Medicare reimbursement for ABPM monitoring in the hospital outpatient department is sufficient for new technology and innovation, but the reimbursement available in the clinic setting is materially lower, to the point that it stifles access and innovation. Providing relatively equivalent reimbursement in the physician clinic setting as currently available in the hospital outpatient setting would improve patient access and compliance with AHA blood pressure monitoring guidelines, resulting in better management of hypertension through services provided in a more appropriate setting of care.

a. What are the current obstacles?

As noted above, the current Medicare policies governing ambulatory blood pressure monitoring (ABPM) services unnecessarily restrict access to this cost-effective service that can dramatically improve health outcomes, reduce chronic medication use, and drive significant economic benefits. The Medicare National Coverage Determination requires an in-person physician visit to set up the test, burdening physician, patients, and clinical staff when a remote visit is appropriate.

In addition, significantly lower reimbursement in the physician office setting, as compared to the hospital outpatient department, also prevents adoption in rural areas and among beneficiaries without ready access to a hospital.

b. What information should providers share with patients when using digital products in the provision of their care?

c. What responsibilities do providers have when recommending use of a digital product by a patient?